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On all sides, the relief was obvious. Last week, the poisonous, 2 ½-year feud that pitted internationally acclaimed blood researcher Dr. Nanni and power of Toronto's Hospital for Sick Children ended in a face-saving compromise. Olivieri, who consistently refused to back down in the experimental drug she was testing, was demoted on Jan. 7 from her position as head of the hemoglobinopathy program at Sick Kids. In exchange, Olivieri, who is also head of a similar program for adults at the Toronto Hospital and a professor at the University of Toronto, agreed to move Sick Kids to the Toronto Hospital next door. The manoeuvre means that Olivieri, who is looking for a cure for sickle cell anemia and thalassemia blood disorders - will continue to have full responsibility for the care of about 450 children and adults at both hospitals. Olivieri, 44, is clearly

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If the high-profile standoff was unusually acrimonious and protracted, its resolution was stunningly swift. After the demotion, the University of Toronto essentially, the professors' union - called on University of Toronto president Robert Prichard to step in. During a marathon negotiating session in the university's offices, the two sides remained in separate rooms with their lawyers while Prichard, the university's lawyer John Murray, and two world-renowned blood disorders specialists shuttled between them. Throughout the day and evening of Jan. 25, advice from David Nathan, head of Harvard Dana-Farber Cancer Institute, and Peter Weatherall of the University of Oxford, became a crucial part of the three-page agreement that was finally signed at about 1:30 a.m. on Jan. 26.

The two emphasized the importance of Olivieri's research and advised the hospital on the best way to resolve the dispute. But according to the lawyers, "Prichard was the guy who made it work. He kept appealing to everyone's better natures." Characterizing the confrontation as "unpleasant," Ruby predicted that the worst is over. Everyone involved has an "obligation they feel keenly," he said, "to the community, to the patients and the staff. We hope, will carry them through."

The agreement is clearly aimed at repairing a badly frayed relationship. As well as reinstating Olivieri, the hospital has agreed to pay her legal fees of \$150,000, and to cover her costs if she is subsequently sued by Toronto-based Apotex Inc., the manufacturer of the drug she was testing.

medication is ineffective and unsafe. Both sides have agreed not to sue one another over any events prior to the date of the agreement. Six weeks after withdrawing a Jan. 6 letter it sent to Olivieri and three of her colleagues - doctors Brenda Gallie, Helen Chan and Peter Durie - reminding them not to communicate with the media without prior approval. Prichard said that, while such bylaws might be relevant in another situation involving a conflict with the public, the letters were "silly, inappropriate and ineffective" in Olivieri's case and had "absolutely no place in trying to suppress the defining issue" during the negotiations, he said, really revolved around preserving Olivieri's research program. Added Prichard: "This hospital treats thousands of children every year and Dr. Olivieri's work is of international significance. The worst thing for a researcher is to be spending all this."

For both sides, there will be much to get over. In late 1995 and early 1996, Olivieri became concerned that deferiprone, a new drug she was testing, was ineffective. When she took her findings to the company, it insisted she had misinterpreted her results, stopped the clinical trials and threatened to sue her with her conclusions. Although the hospital's research ethics board instructed Olivieri to change the consent form so that patients were made aware of the risks to report her findings to the federal government, Olivieri says the hospital administration failed to offer her moral or legal support in her dissent. In 1996, Olivieri also began to fear that deferiprone might cause dangerous levels of iron to accumulate in the liver. The case made headlines and was published in the influential *New England Journal of Medicine*.

A few matters remain outstanding. A grievance launched by the university's faculty association on behalf of Olivieri, Gallie, Chan and Durie was settled last week's settlement. And the hospital's medical advisory committee is continuing to assess a December report on the case by a three-person panel headed by Naimark, former president of the University of Manitoba. In it, Naimark concluded that Olivieri should have reported her findings sooner to the hospital's ethics board. Olivieri, who has questioned Naimark's objectivity because the University of Manitoba received \$120,000 from Apotex during his tenure, said that, under threats from Apotex, she notified patients within a week of concluding that deferiprone might have harmful side-effects.

The case may ultimately be remembered for its impact on an issue that leading researchers say is critically important: the growing dependence of hospitals and universities on the research dollars supplied by companies. Alan Goldbloom, a pediatrician who is vice-president of academic and clinical research at the hospital, says that the hospital has already taken steps to protect the integrity of its research procedures. The hospital has hired a full-time manager to oversee research contracts between researchers and companies that fund their work, and a second position may be created. The hospital has also convened a committee to review the clinical research process. "The individual dispute was a Sick Kids issue," Goldbloom says, "but the problem is a worldwide issue. Nobody here is alone. People at comparable institutions are looking forward to what we come up with, because everybody is trying to manage a very complex area. We are being put in place, Goldbloom added, so that researchers will not have to "start from scratch with each contract."

Included in last week's settlement terms is a short sentence promising that Olivieri will be granted a paid "mini-sabbatical" of six weeks as the hospital "judges it to be possible." Olivieri is almost sheepish when she admits she has no specific plans for filling the time, except to get back to work on sickle cell anemia and thalassemia. "I must be a very overfocused individual," she says. "I miss it. It's what I haven't been able to do. I have people who tell me that when I am looking at data and it's close to that. There's a real elation to feeling you are moving a field forward - and we haven't felt that way

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